

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

LYNDA COULTER and THOMAS
COULTER

Plaintiffs

- and -

LIBERTY MUTUAL INSURANCE
COMPANY

Defendant

Wayne P. Cipollone and Megan Parry, for
the Plaintiffs

Suzanne Courtlander and Wafik Abadir, for
the Defendant

COURT FILE NO.: 01-CV-206753

AND BETWEEN:

LYNDA COULTER and THOMAS
COULTER

Plaintiffs

- and -

GUILLERMO LAVIGNASSE and TESKEY
CONSTRUCTION COMPANY LIMITED

Defendants

D'Arcy McGoey, for the defendants

HEARD: April 1, 2, 3, 4, 7, 8, 9, and 10,
2003

Archibald J.

A) Overview

[1] The plaintiff, Lynda Coulter, was involved in two motor vehicle accidents on the 401 on May 30, 1999 and January 11, 2001. On consent, the trials were heard together. The plaintiff claims damages for pain and suffering, and loss of competitive advantage or future income arising from both accidents. Her husband, Thomas Coulter, has a *Family Law Act* claim for damages for loss of care, guidance, and companionship.

[2] The principal issues in the two cases are as follows:

- (1) was the plaintiff's major injury to her urethra sustained in the May 30, 1999 accident;
- (2) do the injuries in either of the two accidents come within the *Insurance Act* exceptions (this issue is commonly known as whether the injuries have crossed the threshold); and
- (3) if so, the quantum of damages, if any, to be awarded to the plaintiffs.

B) Background Evidence

The First Accident:

[3] At the outset, it is important to point out that liability is not largely contested in either of the two accidents. On May 30, 1999, Ms. Coulter was a passenger in the family Honda Civic when it was unexpectedly cut off on the 401 near Bowmanville by a red Cavalier. Neither the licence of the red car nor its driver has ever been ascertained. When their car was cut off, Mr. Coulter, who was driving, braked, veered, but lost control of the Honda. The car hit the concrete median and flipped around. Fortunately, no other car hit them. I can find no fault with Mr. Coulter's conduct. There was no time for him to react in a manner other than in the way that he did.

[4] The unidentified red Cavalier did not stop. Some individuals who witnessed the accident pulled over and assisted the plaintiffs.

The Injuries:

[5] The Coulters did not go to the Bowmanville hospital. They were taken to their home in Scarborough by the OPP. They then proceeded to the Scarborough Centenary Hospital. Mr. Coulter's injuries were not significant. Ms. Coulter had bruises to her shoulder, breast area, and pelvic area. She was off work for one week. She visited her family physician, Dr. Kilnins, on June 1, 1999. She saw him fourteen times prior to the second accident. Her main complaints arising from the first accident were bruising to the abdominal area, pain in the left knee, and

bruising to the shoulder and neck. Because of her abdominal bruising and pain, a urinalysis was conducted. The results indicated a urinary tract infection.

[6] Through physical therapy and exercise, the majority of the stiffness and pain to her shoulder, neck, and knee had disappeared prior to the second accident. In cross-examination, Ms. Coulter acknowledged that she was not having any significant lower back problems by the time of the second accident. By the end of physical therapy, her neck issues had resolved. Her left knee injury had largely improved by the second accident. The only ongoing issue was the alleged injury to her bladder area.

[7] Most notably, she was having recurrent urinary tract symptoms and infections after the first accident. Her complaints surrounded urinary frequency and urgency. She was sent to two specialists by Dr. Kilnins, namely, Dr. Pizsel, a urologist, and Dr. Easton, a uro-gynecologist.

[8] Dr. Easton conducted a cystourethroscopy on January 30, 2001. In that operation, he also dilated Ms. Coulter's urethra which was narrower than normal. He prescribed Detrol, a prescription drug, to curb Ms. Coulter's symptoms.

[9] By the spring of 2001, Dr. Easton concluded that Ms. Coulter's voiding pattern had considerably improved with the use of the drug. He did not see her again until March 5, 2003. In the spring of 2003, Ms. Coulter again had serious complaints of urinary urgency and frequency. At that time, Dr. Easton concluded that a further cystourethroscopy would be undertaken in the summer of 2003. In his April 1, 2003 letter, Dr. Easton concluded that "Unfortunately, the cause of the recurrent urethral stenosis [narrowing] is very seldom clear." He also stated that the narrowing of the urethra could be related to her urinary tract infections.

The Second Accident:

[10] On January 11, 2001, Ms. Coulter was travelling on the eastbound 401 in the collector lanes between Avenue Road and Yonge Street. The traffic was bumper to bumper and stop and go. Out of nowhere, Ms. Coulter testified that she was plowed in her rear end by a cement truck. She had no warning of the accident. She asserted that her whole body went forward and then backwards in the car. Fortunately, her body did not strike any part of the inside of the car. Her vehicle's trunk was damaged as a result of the rear-end collision.

[11] At discovery, the defendant, Mr. Lavignasse, testified that he was travelling about 50 kilometers per hour prior to the accident. The traffic had just begun to move. When he noticed that Ms. Coulter's car had not begun to pick up speed with the traffic, he began to slow down. At trial, he testified that when he hit Ms. Coulter's car, he was travelling at approximately 5 kilometers per hour. The limited damage to the trunk of the car, the stop and go traffic, and the fact that Ms. Coulter testified that her body did not come into collision with any of the interior parts of her car would support the proposition that Mr. Lavignasse was not travelling at a very large rate of speed when the accident occurred.

[12] I can find no fault on the part of Ms. Coulter in this accident. She was rear-ended without warning. In my view, the fact that she was driving slowly and cautiously in the stop and go traffic does not lead me to conclude that she was contributorily negligent for the accident.

The Injuries From The Second Accident:

[13] Ms. Coulter complained of a stiff neck, pains to her shoulder and back, and headaches as a result of the second accident. She again received physical therapy. She did not take any time off work after the second accident. She testified that her injuries began to improve with physical therapy. By the time of her discovery on July 13, 2001, the injury to her knee had improved to the point that it was actually better than just before the second accident. She still had some residual neck and shoulder pain as of July 13, 2001 although the stiffness and pain were improving. Most importantly, she indicated that the second accident did not affect her urinary condition. It did not aggravate it in any way.

[14] She also indicated that her headaches had become more severe after the second accident. In fact, the severity of her headaches increased about a year and a half after the second accident. By August 2002, Dr. Kilnins had prescribed the prescription drug Maxalt. She advised Dr. Kilnins that that drug helped “100%” with her headaches. Dr. Kilnins made a medical note dated January 13, 2003 which stated that Maxalt worked well on her migraines.

C) Issue No. 1: The Cause of the Urinary Injury

[15] Counsel for Liberty Mutual have argued that there is no evidence to support the proposition that the symptoms and injuries complained of by Ms. Coulter to her urinary tract were caused by the first accident. In my view, based upon the totality of the evidence, I am satisfied that her complaints are directly related to the injuries sustained in the first motor vehicle accident. In reaching that conclusion, I am mindful of the principles of causation including the analysis found in *Athey v. Leonati*, [1996] 3 S.C.R. 458 (S.C.C.). My conclusion is based upon the following evidentiary factors:

- 1) Ms. Coulter sustained a significant contusion to her abdomen as a result of the accident. Her family physician, Dr. Kilnins, stated that it was his “impression” that her recurrent urinary tract symptoms and infections were a direct result of the motor vehicle accident. He concluded in his January 15, 2001 report, that “Ms. Coulter sustained a significant contusion from the seat belt directly above her urinary bladder.” Although Dr. Kilnins is a family physician and not a specialist, he had the opportunity to observe the injuries that Ms. Coulter sustained. He also had the benefit of seeing her on 13 other occasions prior to the second accident. Although his conclusion is only an impression, and is not based upon a review of Dr. Easton’s procedures, in my view, some weight should be accorded to his evidence given his intimate knowledge of Ms. Coulter’s complaints.

2) Ms. Coulter testified that immediately after the accident, her urinary symptoms and problems began to occur. I have carefully assessed her evidence. In my view, Ms. Coulter was a credible and trustworthy witness. She did not attempt to embellish her evidence. She was quick to acknowledge that many of her injuries had improved with physical therapy and exercise. She is a very religious person who, in my view, recognized the importance of her oath-taking. Even though there were occasions when she had to be reminded of her testimony at discovery in order to refresh her memory, she was quick to acknowledge her prior assertions and to adopt them. In the end, I had no reason to doubt the integrity of her testimony.

3) Although Ms. Coulter had suffered from bladder infections in the past, she had not had a bladder infection in the ten years prior to the first accident. Her evidence was corroborated by the clinical record of Dr. Sax (Exhibit 22). Ms. Coulter was symptom free prior to the May 30, 1999 accident. As of June 1, 1999, she complained of pain to her abdomen caused by the seat belt. Her problems with urinary frequency and urgency directly arose after the accident. Her complaints led Dr. Kilnins to refer her to an urologist.

4) Of minor weight to my conclusion is the consultation report of the urologist, Dr. Pizel, dated October 1, 1999. Although his report does not contain a specific reference to the cause of Ms. Coulter's lower urinary tract problems, Dr. Pizel in the consultation history refers to Ms. Coulter's narration of those problems as follows: "She is a 47 year old woman who has had some problems with her lower urinary tract since a car accident. She had some kind of non-specific seat belt injury and she said she has been having infections since then. She complains of pressure and heaviness in the supra pubic area." Dr. Pizel is a specialist in urology. He was consulted in order to assist Ms. Coulter to deal with her symptoms. In my view, if Dr. Pizel was skeptical about the cause of the lower urinary tract problem, he could have indicated his position in his response. The fact that he has reiterated Ms. Coulter's claim at face value is of some minor support to my conclusion.

5) The evidence that is most notably in favour of the plaintiff is the report prepared by the urologist Dr. Ara Keresteci dated August 22, 2002. Dr. Keresteci interviewed Ms. Coulter at the request of Liberty Mutual. Dr. Keresteci has been a urologist for 40 years. He is presently a senior surgeon in the department of urology at St. Michael's Hospital. In my view, he was an impressive witness. Although he acknowledged that he only interviewed Ms. Coulter and did not have an opportunity to review Ms. Coulter's clinical history nor Dr. Easton's file, his report sets out the following conclusions:

The history that she gave me is very suggestive of a pelvic haematoma as a result of the motor vehicle accident. Her bladder function has changed, which may be because of (sic) the bleeding around the bladder. A direct injury did not occur, since she had no bleeding or difficulty in voiding, the haematoma may well have affected the bladder's ability to contract and

empty. As a result of this, she is not only having the bladder symptoms of urgency, frequency but she also is having urinary tract infection because of the poor emptying bladder which becomes very susceptible to infection.

...

The injuries that she sustained as a result of this motor vehicle accident was a pelvic haematoma affecting the bladder, although not directly injuring the bladder. I don't think that there was any fracture of the pelvis.

Dr. Keresteci was called by the plaintiffs at trial. He did not resile from his report. He acknowledged that his report was not based upon a review of the medical history of Ms. Coulter. He also acknowledged that Ms. Coulter's complaints could relate to urethra stenosis. Notwithstanding that acknowledgement, at no time in his evidence did he assert that his opinion in his report was incorrect or no longer valid. At the same time, no counsel on either side of the case suggested to him that his report was inaccurate.

6) Dr. Keresteci has concluded that the haematoma from the accident may well have affected the bladder's ability to contract and empty. He also indicated that Ms. Coulter is having symptoms of urgency, frequency, and infections because of the accident. In my view, whether or not the frequency and urgency problems are related to urethra stenosis or the bladder is immaterial. The urethra is interconnected to the bladder. The bladder is the vessel upon which urine is stored. The urethra is the canal through which urine is emptied from the human body. Dr. Keresteci's report does not limit the injuries sustained by the plaintiff to merely the bladder itself. Ms. Courtlander's and Mr. Abadir's argument that the plaintiff has failed to prove that the urinary stenosis was caused by the motor vehicle accident because the urethra and its stenosis are separated from the bladder is not persuasive in light of Dr. Keresteci's general conclusions. I do not read Dr. Keresteci's report nor understand his evidence at trial to exclude urethra stenosis from his conclusion that the plaintiff's urinary ailments were caused by the car accident.

7) Dr. Easton's analysis does not detract from my conclusion. He has stated that the cause of recurrent urethra stenosis is very seldom clear. In my view, his evidence is largely neutral. It must, however, be recognized that given the fact that Ms. Coulter has the burden of proof on the issue of causation, Dr. Easton's evidence by itself does not advance the plaintiff's case.

[16] In conclusion, I am satisfied on a balance of probabilities that the plaintiff's urination problems are a direct result of the injuries that she sustained from the first motor vehicle accident. The haematoma to the abdomen was sustained as a result of her seat belt and has directly led to her urinary problems and to the urethra stenosis.

D) Issue No. 2: Whether or Not Her Injuries From The First Accident Come Within the Insurance Act Exceptions

[17] Section 267 of the *Insurance Act* immunizes the owner and occupants of motor vehicles and persons present at the scene of the accident from actions by injured parties for loss or damage arising out of motor vehicle accidents unless as a result of the use or operation of the automobile, the injured person had died or has sustained, (b) permanent serious impairment of an important physical, mental, or psychological function. This section is applicable to claims for non-pecuniary losses as a consequence of accidents occurring between November 1, 1996 and the present. It requires that the plaintiff has sustained a permanent serious impairment of an important physical, mental, or psychological function.

[18] In *Ahmed v. Challenger*, [2000] O.J. No. 4188 (S.C.J.), Mr. Justice Jennings confirmed the application of the reasoning of the Ontario Court of Appeal in *Meyer v. Bright* (1993), 15 O.R. (3d) 129 (“*Meyer*”) to the application of the provisions of s. 267 of the *Insurance Act*. Although *Meyer* was decided before the passage of the amendments at issue in this case, it still serves to define the degree and nature of the injury that must be shown to exempt the plaintiff from the no-fault provisions of the *Act*. Thus, a trial judge must answer three sequential questions: (1) Has the injured person sustained a permanent impairment of a physical, mental, or psychological function?; (2) If the answer to question #1 is yes, is the function which is permanently impaired an important one?; (3) If the answer to question #2 is yes, is the impairment of the important function serious?

[19] Counsel for Liberty Mutual have argued that the injuries sustained by Ms. Coulter are not permanent. They concede that whatever injury she has sustained to the urethra or the bladder is to an important function of the human body. They have also argued that the impairment is not serious.

[20] Ms. Coulter has complained regularly about her urination problems. Although she is able to work full-time with some difficulty, she has had to cut back on her recreation activities with her husband, and can not fulfill the responsibilities of her church as much as she would like. Prior to the first accident, she regularly knocked on residential doors on behalf of her church. She is now only able to perform those duties on an intermittent basis. She has indicated that her voiding problems have affected her daily living. In essence, although she is able to function reasonably well on a daily basis, the symptoms are always present.

[21] According to Dr. Easton, the medical procedure of dilating the urethra and the prescription medicine Detrol have significantly curbed her symptoms. The dilation took place in January 2001. Nonetheless, as of the spring of 2003, Ms. Coulter has testified that her symptoms are ongoing. A further dilation of the urethra is scheduled for the summer of 2003. Dr. Easton has opined that urethra stenosis “almost invariably recurs within two to three years, and can contribute to recurrence of voiding dysfunction and lower urinary tract infections.” In my view, Dr. Easton’s report dated April 1, 2003 is important evidence on the issue of the permanency of her injury.

[22] The word “permanent” is defined in the Oxford Encyclopedic English Dictionary (Oxford Press: New York, 1991) at p. 1081 as “lasting, or intended to last or function,

indefinitely.” The word “permanent” which was used in s. 266 of the original Ontario Motorists’ Protection Plan Legislation was interpreted in the case of *Bos v. James*, [1995] O.J. No. 598. In *Bos*, Mr. Justice Howden stated that the word “permanent” does not necessarily mean strictly forever until death; rather, “permanent” is defined as “lasting, or intended to last or function, indefinitely as opposed to temporary.” Howden J. concluded that the term “permanent impairment”... “bears the sense of a weakened condition lasting into the indefinite future without any end limit, as opposed to one predicted to have some defined end.”

[23] Although Dr. Keresteci was optimistic that Ms. Coulter’s bladder problems would ameliorate in a further year or two (as of his report dated August 22, 2002), Dr. Easton’s conclusion which is partly based upon the cystourethroscopy is not as sanguine. His assessment that her condition will almost invariably recur within two to three years is not supportive of the conclusion that Ms. Coulter’s problems will have a defined ending. Rather, Dr. Easton’s analysis is supportive of the determination that her symptoms will carry on indefinitely.

[24] In assessing the duration of her symptoms, I prefer the evidence of Dr. Easton over Dr. Keresteci in these circumstances. Dr. Easton performed the cystourethroscopy and has had the benefit of that analysis to support his conclusion. As previously stated, Dr. Keresteci did not have the opportunity to perform any physical tests or procedures upon Ms. Coulter; thus, his optimistic conclusion has to be given less weight in the circumstances of this case. In my view, Ms. Coulter has sustained a permanent impairment of a physical function.

[25] I must then determine whether the function that is permanently impaired is an important one to Ms. Coulter. As the Court of Appeal has stated in *Meyer*, the use of the word “important” is intended to differentiate between those bodily functions which are important to that particular injured person and those which are not. It cannot seriously be argued that the urination problems sustained by Ms. Coulter are not part of an important function. The answer to question #2 is therefore yes.

[26] The last question is whether the impairment of the important function is serious. Whether the impairment of the important bodily function is serious or not relates to the seriousness of the impairment and not to the injury. For authority for that principle, please see *Meyer* at p. 141, and the decision of *Mohamed v. LaFleur-Michelacci*, [2000] O.J. No. 2476. Ms. Coulter has testified that her symptoms are significant and embarrassing. She has had to curtail her recreation activities and her church responsibilities. Although she is able to work full-time, her urinary symptoms have been more than a nuisance on a regular basis. In my view, the seriousness of the impairment to an important bodily function is obvious.

[27] In *Chilman v. Dimitrijevic* (1996), 28 O.R. (3d) 536 (C.A.), Austin J.A. stated at p. 541:

To summarize, in establishing whether a plaintiff has met the requirements of s. 266 (3) with respect to personal injuries, whether at trial or on a pre-trial motion: (a) the onus is on the party alleging the injury to bring himself or herself within the statutory exception. (b) with respect to past or existing matters, the standard

of proof is upon a balance of probabilities. (c) with respect to what will happen in the future, a party can satisfy the onus by showing upon expert or cogent evidence that there is a substantial possibility that a particular event or condition may occur.

[28] In my view, the plaintiff has satisfied the onus on a balance of probabilities that she has sustained a permanent serious impairment of an important physical function. In my view, she has established upon the balance of probabilities the entitlement to an exception.

E) Issue No. 3: General Damages

[29] In the accident, the plaintiff suffered bruising to her abdomen, knee, neck, and shoulder area. By the conclusion of her physical therapy which ended in October 1999, her soft tissue injuries had largely subsided. The one ongoing ailment was her urination problems. Counsel for the plaintiffs has submitted that Ms. Coulter should be entitled to general damages in a range between \$60,000.00 and \$100,000.00. Counsel for Liberty Mutual have suggested that the appropriate range in the circumstances would be from \$10,000.00 - \$40,000.00. I have carefully considered all of the cases cited in support of either range. Those cases are as follows:

1. *Donohoe v. Wall*, [1987] B.C.J. No. 507;
2. *Alle v. Bean* [1987] O.J. No. 1357;
3. *Alle v. Bean* [1990] O.J. No. 367 (Ont. C.A.);
4. *Graham v. Dove Enterprises Ltd.*, [1993] B.C.J. No. 1218;
5. *Tucker v. Jamieson*, [1995] B.C.J. No. 2369;
6. *Rimington v. Collingwood General & Marine Hospital*, [1995] O.J. No. 3956;
7. *Coombs v. Hibbert*, [1996] O.J. No. 559;
8. *Ovington v. McElroy*, [1996] O.J. 4316;
9. *Liebrecht v. Egesz*, [1999] M.J. No. 85;
10. *Maryanovich v. Eldson*, [2000] O.J. No. 2342.

[30] The injuries sustained to Ms. Coulter's urethra are significant and ongoing. They have caused significant discomfort, embarrassment, and have affected the plaintiff's lifestyle. The prognosis by Dr. Easton is that the problem will likely recur every two or three years on an indefinite basis. She will likely require the intrusive dilation procedure in the future. It is important not to minimize the embarrassing and discomforting nature of the plaintiff's symptoms. At the same time, I must recognize the reality that the plaintiff is able to work full-time and is able to cope reasonably well given this persistent problem.

[31] Her soft tissue injuries have largely ameliorated through exercise and physical therapy. The majority of her pain and stiffness was resolved by October 1999 – five months after the accident.

[32] In reviewing the case law, and carefully assessing the nature and gravity of Ms. Coulter's injuries and symptoms, I have concluded that an award of general damages encompassing both her urinary issues and the soft tissue injuries should be assessed in the amount of \$45,000.00.

F) Mr. Coulter's FLA Claim

[33] Mr. Coulter has made an *FLA* claim. He testified that his recreation activities with his wife have been significantly affected as a result of her urinary problems. The main source of activity was their recreation walks. Those walks have been significantly curtailed. In addition, his wife only helps him on an intermittent basis in going door to door on behalf of their church. He has also indicated that he has had to increase the amount of his house work as a result of his wife's disability and also because of the fact that his wife now works full-time.

[34] Mr. Coulter suffers from a lower back problem and a brain tumour. He has only been able to work part-time since 1995. He testified that his relationship with his wife both before and after the accident was and is very positive. In my view, given the positive nature of their relationship and the extent of Ms. Coulter's disability, Mr. Coulter's claim for loss of care, guidance, and companionship is not all that significant. Counsel for the plaintiffs has submitted that Mr. Coulter's *FLA* claim should be assessed in the amount of \$20,000.00. Both Ms. Courtlander and Mr. McGoey have submitted that the appropriate range is between \$2,500.00 and \$5,000.00. In my view, an appropriate award in these circumstances would be in the amount of \$5,500.00.

G) Whether The Injuries From Accident #2 Come Within The Insurance Act Exceptions

[35] Based upon a review of the evidence in this case, it is my conclusion that the injuries sustained by Ms. Coulter in the January 11, 2001 accident do not constitute a permanent serious impairment of an important physical, mental or psychological function. I have reached that conclusion based upon the following significant factors:

1. The only significant injury of a long-lasting duration from which Ms. Coulter suffers is the urinary problem. Ms. Coulter candidly acknowledged that the second accident did not affect her urinary condition. She stated that it did not aggravate her condition in any way.
2. The impact of the second accident upon Ms. Coulter was, fortunately, not all that severe. At the time of the accident, the traffic was moving slowly. The evidence of Mr. Lavignasse was that he was travelling approximately five kilometers per hour when the accident happened. The extent of the damage to Ms. Coulter's trunk would support that proposition. In addition, Ms. Coulter candidly conceded that her body did not strike the interior of the car after the impact. She complained of pain to her lower back, additional pain to her left knee, and to her shoulder and neck. She also had headaches. She acknowledged that the lower back pain lasted for only a few months after the accident.

3. She acknowledged in cross-examination that if she were experiencing a physical problem than she would have told her family physician, Dr. Kilnins. Dr. Kilnins' notes and the disability certificates that he completed for Ms. Coulter reflect the reality that Ms. Coulter did not make a complaint about her back or neck after April of 2001 with one exception. Dr. Kilnins has recorded one note in August of 2002 where Ms. Coulter complained about headaches and some frontal pain and also pain at the back of her neck. Other than that one note, there was no reference to neck or back pain after April 2001.
4. Her knee injury had improved to the point that it was better than it was before the second accident. She also indicated that she was able to successfully negotiate the stairs in the subway on her way to work.
5. Her headaches seemed to have increased in severity about one and a half years after the second accident. Dr. Kilnins notes the fact that the prescription drug, Maxalt "helped 100% with the headaches" (August 12/02 clinical note). In any event, there is no evidence to support the proposition that the headaches which were bothering her in August of 2002 were at all related to the accident of January 2001 given the year and a half hiatus between her August 2002 complaint and the car accident.
6. One week after the second accident, Ms. Coulter completed an "activities of normal life" occupational form for Liberty Mutual. (Exhibit 11). In cross-examination, Ms. Coulter acknowledged that the information collected in the form was for the benefit of Liberty Mutual. The purpose was to assess her abilities both pre and post the January 11, 2001 accident. A careful review of that form reveals that Ms. Coulter's pre-accident physical abilities and activities were virtually identical to her post-accident activities and abilities. The content is not consistent with the argument that Ms. Coulter has suffered significant injuries from the second accident that have impacted deleteriously upon her life.
7. No evidence was led of any complaints by Ms. Coulter about long-lasting injuries sustained in the second accident. There is no evidence whatsoever of any ongoing injuries from the second accident.

[36] Ms. Coulter has not suffered any injuries of a permanent nature arising out of the second accident. The answer to question #1 is in the negative. As a consequence, I do not need to deal with Questions #2 or #3. Nonetheless, in passing, it is my view that Ms. Coulter has not suffered a serious impairment of any important function. She did not miss any time from work. She is able to work full-time. There is no marked distinction in her physical activities, housekeeping abilities and recreation activities pre or post the January 11, 2001 accident. As a consequence, the answers to Questions #2 and #3 would also be in the negative.

[37] In conclusion, there was no evidence presented to support the proposition that Ms. Coulter has sustained a permanent serious impairment of an important physical, mental or psychological function. The evidence in this case does not support the conclusion that her

injuries fall within an exception set out in section 267.5 of the *Insurance Act*. Mr. Coulter's claim for damages for non-pecuniary loss under clause 61(2)(e) of the *Family Law Act* is also dismissed.

[38] If, however, I am incorrect in the determination that Ms. Coulter has not sustained a permanent serious impairment of an important physical function, I would award Ms. Coulter \$8,500.00 for general damages for her soft tissue injuries. The injuries sustained in the second accident do not overlap nor have they exacerbated the injuries sustained in the first accident. From that general damage award, the statutory deductible pursuant to s. 267.5(7) would then be deducted.

H) Loss of Future Income or Loss of Competitive Advantage

[39] Mr. Cipollone has asserted that Ms. Coulter will suffer a loss of competitive advantage, or, alternatively, a loss of future income on the basis that at some future point in time she will not be able to work full-time as a medical secretary. Until her husband's physical ailments became severe, she had worked part-time for many years as a medical secretary. Because of her husband's part-time employment, she has had to go back to work as a medical secretary on a full-time basis. Mr. Cipollone concedes that her present income on an hourly basis is similar to the income that she earned prior to her husband's disability. He is not claiming damages for loss of past income. He has argued that at some point in the future because of her urinary problems and soft tissue injuries, she will not be able to work full-time until the age of 65; however, no evidence, medical or otherwise, was introduced in support of this claim.

[40] Ms. Coulter has indicated that she is able to work full-time although her bladder issues do cause her discomfort. There was no evidence led that in the future, Ms. Coulter might not be able to work or might have to take a lesser paying job because of her physical condition. The onus is upon the plaintiff to lead evidence in support of her claim for loss of future income or loss of competitive advantage. In my view, if I were to conclude that at some point in the future, Ms. Coulter would not be able to work or would lose some competitive advantage because of her urinary issues, I would be speculating and reaching an arbitrary conclusion on the basis of a vacuity of evidence. Ms. Coulter's claim for loss of future income or loss of competitive advantage must be dismissed.

I) Conclusion

[41] On the basis of the first motor vehicle accident of May 30, 1999, Ms. Coulter is entitled to an award of general damages in the amount of \$45,000.00. The statutory deductible pursuant to section 267.5(7) will be subtracted from that award. Mr. Coulter is entitled to damages for non-pecuniary loss under clause 61(2)(e) of the *F.L.A.* in the amount of \$5,500.00. The statutory deductible will be subtracted from that amount. Ms. Coulter's claim for loss of future income or loss of competitive advantage is dismissed. Unless there are factors of which I am not aware, the plaintiffs' are entitled to their costs on a partial indemnity basis against the defendant, Liberty Mutual Insurance Company.

[42] In the other action, both plaintiffs' claims have been dismissed. As a consequence, unless there are factors of which I am not aware, the defendants, Guillermo Lavignasse and Teskey Construction Company Limited, are entitled to their costs on a partial indemnity basis.

[43] All of the parties are encouraged to resolve the issues of costs among themselves. If, however, they are unable to do so, the parties are required to file written submissions within 30 days of the release of this judgment.

Archibald J.

Released: May 21, 2003

COURT FILE NO.: 00-CV-196435 and 01-CV-206753
DATE: 20030521

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

LYNDA COULTER and THOMAS COULTER

Plaintiffs

- and -

LIBERTY MUTUAL INSURANCE COMPANY

Defendant

AND BETWEEN:

LYNDA COULTER and THOMAS COULTER

Plaintiffs

- and -

GUILLERMO LAVIGNASSE and TESKEY
CONSTRUCTION COMPANY LIMITED

Defendants

REASONS FOR JUDGMENT

Archibald J.